





OBOMOCARE

Community Based Organization

Gusii Region, Kenya

Delivering Care.

INTEGRATED COMMUNITY SUPPORT PROGRAMME

International Donor Funding Proposal

Funding Requested: USD 1,801,697 over 24 Months

Families Served: Over 2,000 Households in Kisii and Nyamira Counties

Established: 2020 | Status: Registered CBO, Kenya

Submitted to: International Donors, Partner Organizations and Global Well-wishers

Contact: obomocare@gmail.com | Kisii and Nyamira Counties, Kenya

June 2026

Table of Contents

1. Introduction	5
2. Situation Analysis	6
2.1 The Gusii Region.....	6
2.2 Poverty and Food Insecurity.....	6
2.3 Healthcare Access and Mobility.....	7
2.4 Caregiving Gaps and the Dignity Deficit	7
2.5 Social Isolation	8
2.6 Community Health Volunteer Coordination	8
3. Problem Statement	10
4. Theory of Change.....	12
5. Project Description	14
5.1 Programme Overview.....	14
5.2 Target Beneficiaries.....	14
5.3 Programme Pillars and Activities	14
5.4 Volunteer Caregiver Corps.....	15
5.5 Digital Coordination System	15
6. Logical Framework	17
7. Organisational Profile and Institutional Capacity	19
7.1 History and Identity	19
7.2 Governance.....	19
7.3 Staffing and Volunteers	19
7.4 Track Record	20
8. Sustainability Plan.....	21
8.1 Revenue Diversification	21
8.2 Volunteer Retention.....	21
8.3 Evidence and Advocacy	22
9. Risk Management.....	23
10. Budget Framework.....	24
11. Monitoring, Evaluation and Learning	26
12. Funding Request and Donor Partnership.....	28

Executive Summary

OBOMOCARE is a registered Community Based Organization operating in the Gusii Region of southwestern Kenya, serving Kisii and Nyamira Counties since its founding in 2020. Over the past five years, the organization has provided integrated community support to over two thousand vulnerable households, addressing the intersecting challenges of food insecurity, limited access to healthcare transport, inadequate personal caregiving, and social isolation among the elderly, the chronically ill, and persons living with disabilities.

The organization was established in direct response to conditions observed at the household level within the Gusii community. Families were struggling not with one problem but with several problems simultaneously, and the absence of any coordinating support structure meant that individuals fell through every available gap. OBOMOCARE was founded to close those gaps through a model built on community presence, volunteer care, and practical daily support.

From 2020 to the present, OBOMOCARE has operated entirely through local and community-generated resources. Funding has come from personal contributions by friends and local well-wishers, in-kind donations from supermarkets and local businesses, support from church and faith community networks, community group fundraising, and Harambee initiatives. These resources, while limited in scale, have sustained a proven and respected program that is now ready for structured growth.

OBOMOCARE is seeking international funding of USD 1,801,697 over a period of twenty-four months. This funding will enable the organization to expand its reach from its pioneer households to one thousand five hundred enrolled beneficiary households, formalize and train OBOMOCARE corps of forty volunteer caregivers, establish a digital coordination and monitoring system, and build the organizational infrastructure necessary for sustainable, evidence-based operation at ward and ultimately county level.

This proposal sets out the context of need within the Gusii Region, the organizational history and capacity of OBOMOCARE, the program design and theory of change, the logical framework, the sustainability strategy, the risk management approach, and a detailed budget framework. It is submitted with the confidence that a modest, well-targeted investment in this organization will produce measurable, lasting improvements in the lives of some of Kenya's most vulnerable people.

1. Introduction

This proposal is submitted by OBOMOCARE, a Community Based Organization registered in Kenya and operating within the Gusii Region, which encompasses Kisii and Nyamira Counties in the southwestern part of the country. The Gusii Region is home to the Abagusii community, one of Kenya's distinct ethnic communities with a population of nearly 1.9 million people distributed across two counties of highly fertile but economically constrained highland terrain.

OBOMOCARE was founded in 2020 by individuals who lived and worked within this community and observed at close range the compounding difficulties facing its most vulnerable members. The organization set out with a clear purpose that is to provide practical, dignified, and integrated support to households that were being left behind by formal service systems. This meant not choosing between providing food or facilitating transport or offering personal care, but doing all of these things together, because the needs themselves do not arrive separately.

Over five years of operation, OBOMOCARE has grown from a small, informal network of volunteers and community supporters into a structured organization with governance systems, a trained volunteer base, documented beneficiary records, and an established track record of service delivery across Kisii and Nyamira Counties. The organization now stands at a point where its model has been proven and its impact is visible, but where continued growth requires a level of institutional and financial support that only international partnership can provide.

This proposal presents the full case for that partnership. It is written for international donors, development partners, and global well-wishers who wish to direct their resources toward community-led organizations that are already working, already trusted, and already producing results. OBOMOCARE does not ask to be built from nothing. It asks to be supported to scale.

2. Situation Analysis

2.1 The Gusii Region

The Gusii Region comprises Kisii County, with a population of 1,266,860 people,¹ and Nyamira County, with a population of 605,576 people,² giving a combined regional population of approximately 1.9 million. Both counties are located in the former Nyanza Province in southwestern Kenya and are inhabited predominantly by the Abagusii people. The region is characterized by dense rural settlement, smallholder agriculture, and a relatively high population density by Kenyan standards.

Despite the region's agricultural productivity and cultural cohesion, significant portions of the population live in conditions of chronic poverty that limits their access to health services, adequate nutrition, and dignified daily living. These conditions are most acute among elderly persons living alone, households headed by orphaned children, persons living with chronic illness including HIV and non-communicable diseases, and individuals with physical disabilities who lack caregiving support.

2.2 Poverty and Food Insecurity

The Kenya National Bureau of Statistics Poverty Report based on the 2019 Kenya Continuous Household Survey recorded a food poverty headcount rate of 39.7 percent in Kisii County and 35.9 percent in Nyamira County.³ These figures are significantly above Kenya's national food poverty rate and reflect a structural food insecurity problem that is not seasonal but persistent. Nationally, approximately 25 percent of Kenyans live below the international poverty line as of 2023 and 2024,⁴ but in rural Gusii households, the burden falls disproportionately on the most vulnerable members of the family unit.

Within the households that OBOMOCARE serves, food insecurity takes specific and painful forms. Elderly persons living alone may go without food for extended periods because they

¹Kenya National Bureau of Statistics. 2019 Kenya Population and Housing Census, Volume III. <https://www.knbs.or.ke/2019-kenya-population-and-housing-census-reports/>

²Kenya National Bureau of Statistics. Nyamira County Population, 2019 Census. <https://www.knbs.or.ke/2019-kenya-population-and-housing-census-reports/>

³ KNBS. The Kenya Poverty Report, 2019 KCHS. Table 3.4: Food poverty headcount by county <https://www.knbs.or.ke/wp-content/uploads/2023/09/The-Kenya-Poverty-Report-2019.pdf>

⁴Wikipedia / World Bank. Poverty in Kenya: 25% of Kenyans below international poverty line, 2023/24. https://en.wikipedia.org/wiki/Poverty_in_Kenya

cannot cook for themselves and have no family member nearby to assist them. Households caring for a chronically ill person face the dual burden of reduced income and increased expenditure, often resulting in inadequate nutrition for both the patient and other household members. Children in such households frequently go to school without a meal.

2.3 Healthcare Access and Mobility

Research published on geographic accessibility to health facilities in Kenya confirmed that access to healthcare within one hour of travel varies dramatically across counties, with rural communities in western Kenya among those facing the highest barriers.⁵ In Kisii and Nyamira Counties, the barriers to healthcare access are not primarily about the existence of facilities but about the cost and physical difficulty of reaching them.

Motorcycle taxis, known locally as boda bodas, are the most common means of transport to health facilities in rural Gusii. For households in poverty, the cost of a boda boda journey to a clinic and back represents a significant portion of daily income. For elderly persons, persons with mobility impairments, or those recovering from illness, even a short journey on a motorcycle is physically challenging or impossible without assistance. The result is that clinic appointments are missed, chronic disease management breaks down, medications run out, and conditions that could be controlled become life-threatening.

Kenya records more than 5,000 maternal deaths and 35,000 infant deaths annually, with a significant proportion attributed to delayed access to health facilities in rural communities.⁶ While OBOMOCARE's primary focus is not maternal health, these figures reflect the broader failure of transport support systems that affects all categories of health-seeking behavior in the communities we serve.

2.4 Caregiving Gaps and the Dignity Deficit

One of the most invisible dimensions of vulnerability in the Gusii Region is the caregiving gap. As working-age adults migrate to urban centers in search of employment, elderly parents are increasingly left behind in rural homes without adequate care. For those who are elderly,

⁵Macharia PM et al. Geographic accessibility to public and private health facilities in Kenya in 2021. PLOS Medicine. <https://pmc.ncbi.nlm.nih.gov/articles/PMC9670107/>

⁶The Star / UNICEF estimates. Kenya: 5,000+ maternal and 35,000 infant deaths annually, largely in rural communities. <https://www.the-star.co.ke/counties/nyanza/2026-05-25-special-edition-page-9-story-1-women-riders-breaking-barriers-to-save-mothers-in-rural-kenya-1>

frail, or living with conditions that affect their physical mobility or independence, the absence of daily personal care has immediate and severe consequences.

Personal care in this context means the basic assistance that a person needs to maintain hygiene, dignity, and physical wellbeing: bathing, dressing, grooming, wound care, repositioning for those who are bedridden, and general support with daily activities. Without this care, individuals develop preventable hygiene-related conditions, experience psychological deterioration from shame and helplessness, and become increasingly isolated. OBOMOCARE has made restoring dignity through personal care a central and non-negotiable element of its model, because dignity is not a luxury. It is a right. Restoring dignity has also been made possible through partnerships with churches who provide youth and women groups to give care to the identified families.

2.5 Social Isolation

Social isolation among elderly and chronically ill persons in rural Kenya is a serious and under-addressed problem. Many of the individuals OBOMOCARE serves have no regular visitor, no person to converse with, and no way to participate in community life. This isolation is not merely an emotional inconvenience. It accelerates cognitive decline, deepens depression, reduces motivation to seek or adhere to treatment, and creates a cycle of withdrawal that makes all other forms of vulnerability worse.

OBOMOCARE's response to social isolation goes beyond checking in on beneficiaries. Volunteer caregivers are trained to provide meaningful companionship through structured conversation, games, storytelling, and accompaniment to social events and community opportunities. The organization has observed repeatedly that when isolated individuals regain a sense of connection and visibility, their overall health and wellbeing improves measurably, even before other interventions take effect.

2.6 Community Health Volunteer Coordination

Kenya's Community Health Policy 2020 to 2030 designates Community Health Promoters as the first level of the national health system, responsible for home visits, health promotion, referrals, and community-based case management.⁷ In Kisii and Nyamira Counties, CPVs are active across most communities but face persistent challenges including limited logistical

⁷Government of Kenya. Kenya Community Health Policy 2020-2030. CHVs designated as first level of health system. <https://chwcentral.org/kenyas-community-health-volunteer-program/>

support, high attrition due to the unremunerated nature of their work, and insufficient coordination with other community support structures.

OBOMOCARE operates in close coordination with CHPs in the sub-counties where it works. The organization provides the transport coordination, food support, and referral documentation that enables CHVs to follow through on the cases they identify. This partnership is mutually reinforcing and is central to the reach and effectiveness of OBOMOCARE's model.

3. Problem Statement

The challenge that OBOMOCARE was established to address is not a single problem but a cluster of interconnected problems that compound one another in the lives of vulnerable households. Understanding this interconnectedness is essential to understanding why an integrated response is required.

Food insecurity weakens the body and reduces immunity. A weakened body is more susceptible to illness and less able to recover from it. Illness increases the need for healthcare, but mobility barriers make healthcare inaccessible. Missed clinic visits lead to treatment interruption. Interrupted treatment leads to worsening of chronic conditions. Worsening conditions increase the caregiving burden on family members. Where no family member is available to provide care, the individual deteriorates in isolation. Isolation deepens depression and withdrawal. Depression reduces the individual's motivation to seek help, attend social functions, or engage with community health structures. The cycle is self-reinforcing and, without external intervention, self-sustaining.

The problem is further compounded by the structural absence of formal support systems at the household level. Kenya's formal health system operates primarily at facility level. Social protection programs exist but do not reach the majority of households in the categories OBOMOCARE serves. OBOMOCARE Community Health Promoters, while present and committed, lack the logistical and material support needed to address the full range of needs they encounter. The result is a community of vulnerable individuals who are known to be in need but not being adequately reached.

OBOMOCARE's Integrated Community Support Model was designed specifically to address this cluster of problems simultaneously. The four pillars of the model, namely food support, transport facilitation, personal dignity care, and meaningful companionship, correspond directly to the four dimensions of the problem described above. Each pillar reinforces the others, and together they create a comprehensive support environment that no single-pillar intervention can replicate.

The organization submits that addressing these four interconnected problems within a coordinated, household-level program is not only the most effective approach but the only approach that can produce lasting improvement in beneficiary wellbeing. This proposal seeks

the resources to do exactly that, at a scale that reflects both the extent of need and the capacity of the organization.

4. Theory of Change

OBOMOCARE's theory of change is grounded in the conviction that sustainable improvement in household wellbeing requires simultaneous attention to the multiple, interconnected barriers that prevent vulnerable individuals from living with health and dignity. The model operates on a causal pathway that connects community inputs to long-term outcomes through a structured sequence of activities and intermediate results.

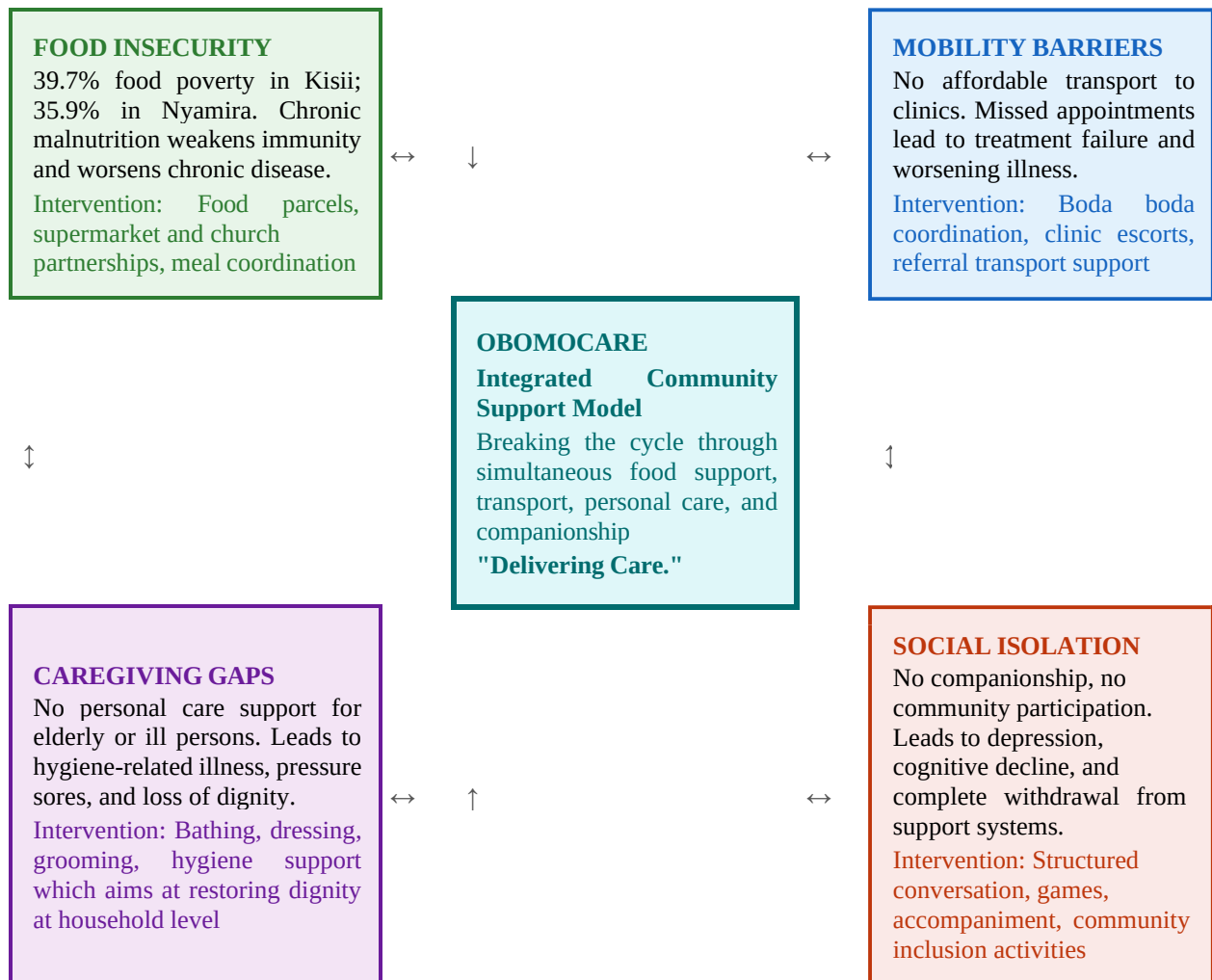
When food insecurity is addressed, individuals have the nutritional baseline needed to benefit from medical treatment. When transport is provided, clinic appointments are attended and medication regimens are maintained. When personal care is delivered, dignity is restored and hygiene-related complications are prevented. When companionship is offered, social isolation is reduced and individuals re-engage with community life. These four streams of support, delivered concurrently and coordinated through a trained volunteer workforce, produce outcomes that no single stream could achieve alone.

The intermediate outcomes, improved clinic attendance, better medication adherence, improved nutrition, restored dignity, and reduced isolation, combine to produce a measurable improvement in overall household wellbeing. This improvement, sustained over time, reduces the incidence of preventable illness, strengthens household resilience, and creates the conditions for the community's own capacity to support its vulnerable members to grow. The long-term impact is a community that is more self-sustaining, better connected to health services, and less dependent on emergency external intervention.

The diagram on the following page illustrates the full theory of change, showing the interconnectedness of the four root problems and the pathway through which OBOMOCARE's integrated response produces lasting impact. The circular nature of the diagram reflects the reality that the problems reinforce one another and that the interventions must equally reinforce one another if the cycle of vulnerability is to be broken.

Figure 1: The Interconnectedness of Vulnerability and OBOMOCARE's Response

Each problem causes and amplifies the others. OBOMOCARE's four pillars interrupt the cycle simultaneously.



The arrows between each problem reflect two-way causation: each problem worsens the others. OBOMOCARE's four pillars interrupt all four simultaneously.

Integrated Support

Improved clinic attendance | Better nutrition | Restored dignity | Reduced isolation

**Improved Household Wellbeing and Community Resilience**

Ward Level → County Level → Regional Scale-Up → National Replication

5. Project Description

5.1 Program Overview

The proposed program is titled the OBOMOCARE Community Link Support Program, Phase Two. It is designed to be implemented over a period of twenty-four months across selected sub-counties of Kisii and Nyamira Counties. The program will enroll one thousand five hundred vulnerable households as primary beneficiaries, with each household receiving the full range of OBOMOCARE services across all four pillars: food support, transport facilitation, personal care, and companionship.

The program will simultaneously strengthen OBOMOCARE's organizational infrastructure, including formalizing its volunteer caregiver corps through structured training and certification, deploying a digital coordination and monitoring system, establishing formal partnerships with boda boda associations and local food suppliers, and building the evidence base for county-level advocacy and scale-up.

5.2 Target Beneficiaries

Primary beneficiaries are vulnerable households in the target sub-counties that meet one or more of the following criteria: households headed by elderly persons aged sixty years and above; households where the head or a primary member is living with a chronic illness including HIV, hypertension, diabetes, or other non-communicable diseases; households where a member is physically disabled and lacks caregiving support; and households classified as food poor based on established Kenya National Bureau of Statistics criteria.

Secondary beneficiaries include the volunteer caregivers themselves, who gain skills, recognition, and a structured support environment; community health volunteers whose coordination capacity is strengthened through partnership; and the wider community, which benefits from a reduction in preventable illness and an increase in social cohesion.

5.3 Program Pillars and Activities

Under the food support pillar, the program will provide OBOMOCARE FOOD BASKET every fortnight to enrolled households, coordinate with local supermarkets and faith communities for additional in-kind food donations, and support meal preparation for elderly persons who cannot cook independently.

Under the transport facilitation pillar, the program will negotiate and maintain formal arrangements with boda boda associations in each target sub-county to provide subsidized clinic transport for enrolled beneficiaries. Transport coordinators will schedule and document all journeys, ensuring that clinic appointments are attended and that post-appointment follow-up is tracked.

Under the personal care and dignity pillar, trained volunteer caregivers will make regular household visits to provide bathing, grooming, dressing, and basic hygiene support to elderly and infirm beneficiaries. Caregivers will be equipped with appropriate hygiene supplies and will document the condition of each beneficiary at each visit to enable early identification of deteriorating health.

Under the companionship pillar, caregivers will spend dedicated time with isolated beneficiaries in structured social interaction, including conversation, games, storytelling, and accompaniment to community events. Beneficiary social engagement will be tracked as a key performance indicator.

5.4 OBOMOCARE Volunteer Caregiver Corps

The program will train and deploy a corps of forty community volunteer caregivers drawn from the target communities. Volunteers will undergo a structured training program covering personal care techniques, safeguarding and dignity principles, basic health literacy, First AID, CPR training, waste management, Basic mental Health training, documentation, and communication skills. Training will be delivered in partnership with a qualified health training institution. Volunteers will receive a monthly support stipend, transport reimbursements, and a clear career pathway including the possibility of progressing to community health assistant roles within the county government system.

Volunteer retention is a central concern of the program design. Evidence from CHV program across Kenya shows that even modest financial support combined with skills recognition and structured supervision significantly reduces attrition.⁸

5.5 Digital Coordination System

⁸Ogutu MO et al. Factors influencing the performance of Kenyan community health volunteers. PLOS Global Public Health, 2023. <https://journals.plos.org/globalpublichealth/article?id=10.1371%2Fjournal.pgph.0001815>

A simple, mobile-phone based coordination and monitoring system will be deployed in the first six months of the program. The system will enable volunteer caregivers to record household visits, flag concerns, and communicate with the Field Coordinator without requiring costly data plans or high-end devices. Program management will use the system to generate monthly activity reports, track against key performance indicators, and prepare quarterly donor reports with accurate and complete data.

6. Logical Framework

The following logical framework summarizes the program's objectives, outputs, activities, and verifiable indicators for the proposed twenty-four-month period.

Level	Statement	Indicator	Target (24 months)	Means of Verification
Goal	Improved wellbeing, dignity, and health outcomes for vulnerable households in Gusii Region	Composite household wellbeing score	70% of enrolled households show measurable improvement	Baseline and endline surveys; Ministry of Health records
Purpose	1,500 vulnerable households receive integrated food, transport, personal care, And companionship support	Households enrolled and completing full-service cycle	1,500 households by Month 24	Beneficiary register; field visit records
Output 1	Food security improved for enrolled householdsg	Percentage of households with adequate food access (two or more meals per day)	From baseline to 80 percent by Month 24	Monthly food registers; household visit records
Output 2	Clinic attendance and medication adherence improved	Clinic appointment attendance rate; medication adherence rate	85 percent appointment attendance rate	CHV referral logs; facility records; volunteer reports
Output 3	Trained volunteer caregiver corps delivering personal care and companionship	Number of trained volunteers; monthly visits per beneficiary household	40 trained caregivers; minimum 4 visits per household per month	Training certificates; visit logs; supervisor reports

Output 4	Digital coordination and monitoring system operational	System live; percentage of visits recorded digitally	System deployed by Month 6; 90 percent digital recording by Month 12	System data exports; IT audit reports
Output 5	Social isolation reduced and community participation increased	Beneficiary social engagement score; number of community events attended	60 percent reduction in isolation scores from baseline	Baseline and endline wellbeing assessments

7. Organizational Profile and Institutional Capacity

7.1 History and Identity

OBOMOCARE was founded in 2020 by a group of individuals from the Gusii community who shared both a diagnosis of the problem and a determination to respond to it. The name OBOMOCARE reflects the Gusii concept of nurturing warmth, the kind of care that a parent offers a child, and it expresses the organisation's core orientation toward dignified, relational, and sustained support for those who are most vulnerable.

From its founding, the organisation operated as an entirely community-funded initiative. It was sustained through personal contributions from friends and local wellwishers, in-kind donations from supermarkets and retail businesses in Kisii and Nyamira towns, regular support from church congregations and faith community networks, community fundraising events including Harambee contributions, and support from local entrepreneurs who believed in the mission. These sources of support, while modest, enabled the organisation to serve over one thousand families between 2020 and 2025 and to build the systems, relationships, and track record that justify this request for international funding.

7.2 Governance

OBOMOCARE is governed by a Board of Trustees comprising five members including community leaders, a healthcare professional, a faith leader, and a financial accountability officer. The Board meets quarterly and is responsible for strategic direction, financial oversight, and organizational accountability. Day-to-day operations are managed by a Program Director, supported by a Field Coordinator who supervises the volunteer network and coordinates field activities.

The organization maintains beneficiary registers, service delivery records, financial records, and volunteer attendance documentation. A safeguarding policy is in place to protect the rights and dignity of all beneficiaries. An anti-fraud policy and procurement procedures have been adopted by the Board. Annual financial reviews are conducted, and the organization is committed to expanding these to full external audits under the proposed program.

7.3 Staffing and Volunteers

Role	Number	Function
Board of Trustees	5 members	Strategic governance, oversight, financial accountability
Program Director	1	Strategic leadership, donor relations, program management
Field Coordinator	1	Volunteer supervision, field logistics, beneficiary records
Community Volunteer Caregivers	22 actives	Household visits, personal care, companionship, food support
Community Health Volunteer Partners	18 linked	Health referrals, household registration, clinic coordination, background checks

7.4 Track Record

OBOMOCARE's track record over five years of operation provides the clearest evidence of its capacity to deliver the proposed program. The following figures represent cumulative service delivery between 2020 and 2025, verified through the organization's internal records.

Service Area	Households Reached	Trend
Food support	1050 plus households	40 percent year-on-year growth
Transport facilitation	325 plus households	35 percent year-on-year growth
Personal dignity care	610 plus elderly persons	55 percent year-on-year growth
Companionship visits	690 plus isolated persons	60 percent year-on-year growth
Total households served (cumulative)	Over 2,000 families	Sustained across all 5 years

8. Sustainability Plan

OBOMOCARE's approach to financial and programmatic sustainability is grounded in a deliberate strategy of diversification. The organization does not intend to remain dependent on any single source of funding, including international donors. The proposed program includes a structured sustainability workplan that will, over twenty-four months, strengthen the revenue base and institutional partnerships necessary for long-term independent operation at scale.

8.1 Revenue Diversification

Revenue Source	Description	Current Status
Local business and supermarket partnerships	In-kind food donations, seasonal campaigns, corporate social responsibility partnerships with regional retailers	Active. Six local partners engaged.
Church and faith network contributions	Regular collections, volunteer mobilization, community outreach facilitation	Active. Four congregations contributing.
Diaspora fundraising	Online fundraising campaigns targeting the Gusii diaspora within Kenya and internationally	Growing. Digital platform to be formalized under proposed program.
Boda boda association partnerships	Negotiated subsidized transport rates for beneficiary clinic visits in target sub-counties	Pilot. Two associations engaged.
County government partnership	Integration into county social protection and community health programs; potential co-funding	Dialogue initiated.
NGO and INGO sub-granting	Positioning OBOMOCARE as a credible implementing CBO partner for larger development organizations working in the region	Prospecting. To be formalized under proposed program.

8.2 Volunteer Retention

The long-term sustainability of OBOMOCARE's service model depends on retaining committed and skilled volunteer caregivers. The program will retain volunteers through a combination of monthly support stipends, skills training and certification, structured

supervision and peer support, and formal recognition mechanisms. The skills gained through the caregiver training program will qualify volunteers for progression within Kenya's community health system, creating a tangible career incentive for sustained engagement.

8.3 Evidence and Advocacy

A critical element of the sustainability strategy is the generation of rigorous program evidence. The digital monitoring system, annual evaluations, and published results will position OBOMOCARE as a data-driven organization capable of informing county and national policy on integrated community support models. This evidence base will attract future donors, facilitate county government partnership, and support the organization's application for larger grants from international institutions.

9. Risk Management

The following table identifies the principal risks to program delivery and sets out the mitigation strategies that OBOMOCARE has adopted or will adopt to manage each risk.

Risk	Likelihood	Impact	Mitigation Strategy
Volunteer attrition	Medium	High	Monthly support stipend, skills training, peer support groups, recognition events, and clear role boundaries to prevent burnout
Food supply disruption	Medium	High	Diversified supplier base of at least four partners; emergency reserve fund equivalent to two months of food costs maintained at all times
Transport cost increases	Medium	Medium	Formal MOUs with two boda boda associations; cash transport reserve; exploration of low-cost bicycle ambulance for last-mile journeys
Donor funding interruption	Low	High	Diversified funding base; operating reserve fund covering three months of costs; phased spending plan aligned to milestones
Community trust concerns	Low	High	Beneficiary feedback system; community liaison committee; monthly open community forums
Safeguarding risk to beneficiaries	Low	Very High	Safeguarding policy in place; mandatory safeguarding training for all volunteers; incident reporting system; Board oversight of all complaints

10. Budget Framework

The following budget framework presents OBOMOCARE's funding request of USD 1,801,697 over twenty-four months. All figures are expressed in United States Dollars and have been calculated based on current field-level costs in Kisii and Nyamira Counties. Administrative costs are maintained below fifteen percent of total program expenditure, consistent with international standards for community-based organizations.

Budget Line	Basis	Unit Cost (USD)	Units	Total (USD)
PROGRAMME COSTS				
Food support packages (1,500 households)	Per household per month	30	1,500 x 24 months	1,080,000
Transport facilitation fund	Per household per month	3	1,500 x 24 months	108,000
Personal care and hygiene supply kits	Per beneficiary per month	7	1,500 x 24 months	252,000
Volunteer training and certification (40 caregivers)	Per person once off	120	1	4,800
Volunteer monthly support stipends (40 caregivers)	Per person per month	140	40 x 24 months	134,400
Digital coordination system setup and maintenance	Lump sum	45,000	1	45,000
Community outreach and beneficiary registration	Per quarter	275	8 quarters	2,200
Program Costs Subtotal				1,626,400
ADMINISTRATION AND STAFFING				
Program Director salary (part-time)	Per month	2,300	24 months	55,200

Field Coordinator salary	Per month	1,500	24 months	36,000
Monitoring, evaluation, and reporting	Per quarter	1,530	8 quarters	12,240
Annual external audit	Per year	20,000	2 years	40,000
Administration Subtotal				143,440
Contingency and emergency reserve (approximately 1.8%)				31,857
TOTAL FUNDING REQUEST				1,801,697

Note: Food support costs are calculated at USD 30 per household per month for 1,500 enrolled households over the first twenty-four months of the programme. Transport facilitation provides two subsidised clinic trips per enrolled household per month at an average cost of USD 3. All figures have been verified against current market rates in Kisii and Nyamira Counties.

11. Monitoring, Evaluation and Learning

OBOMOCARE's monitoring and evaluation framework is designed to ensure that the program is accountable to its beneficiaries, its donors, and the communities it serves. Data collection is built into the daily activities of volunteer caregivers and field staff, enabling real-time tracking of program delivery and early identification of problems.

A household baseline survey will be conducted at the point of enrolment for each beneficiary, capturing data on food security, healthcare access, social engagement, and overall wellbeing. This baseline will be used to measure change over the course of the program. Monthly field visit logs completed by volunteer caregivers will track service delivery, document beneficiary conditions, and flag any concerns for follow-up. Quarterly program reviews will analyze aggregate data and compare delivery against the targets set out in the log frame. A formal midterm evaluation will be conducted at Month 12, and an independent external evaluation will be completed at Month 24.

The digital coordination system, to be deployed in the first six months of the program, will streamline data collection and reporting, reducing the administrative burden on volunteers and ensuring that the Field Coordinator has access to accurate and timely information for program management. Quarterly donor reports will draw on this data to provide a full and transparent account of program delivery, financial expenditure, and beneficiary outcomes.

Learning from program implementation will be systematically documented and shared. OBOMOCARE is committed to honest reporting, including the documentation of what has not worked and what adjustments have been made. The organization believes that this kind of honest learning is essential to its credibility as a partner and to the long-term quality of its service model.

Key Performance Indicator	Baseline (estimated)	Target at Month 24
Percentage of households with adequate food access	Below 50 percent	80 percent or above
Clinic appointment attendance rate	Below 60 percent	85 percent or above
Medication adherence rate among enrolled households	Approximately 55 percent	80 percent or above

Beneficiaries reporting improved dignity and respect	Not yet measured	90 percent or above
Reduction in social isolation score	Baseline established at Month 1	60 percent reduction
Volunteer caregiver retention rate	Approximately 70 percent	85 percent or above

12. Funding Request and Donor Partnership

OBOMOCARE is requesting a total of USD 1,801,697 over a period of twenty-four months to implement Phase Two of its Integrated Community Support Program across Kisii and Nyamira Counties. This funding will enable the organization to scale from its pioneer base of households to one thousand five hundred enrolled beneficiary households, while simultaneously strengthening its organizational infrastructure, volunteer corps, and evidence systems to support sustainable long-term operation.

This is not a request for charity directed at an organization with an untested idea. OBOMOCARE has been operating for five years, has served over two thousand families, and has done so without a single dollar of international funding. Its model has been proven in the field, its relationships within the community are well established, and its governance structures are in place. What is being requested is the investment needed to take a proven model to the scale of impact that the level of need demands.

Donors are invited to partner with OBOMOCARE in any of the following ways. A full program partnership covering the total request of USD 1,801,697 would provide the complete funding package required for all four program pillars and the organizational strengthening components. Pillar-specific contributions are equally welcome and would be directed in full to the relevant program component, including food support at USD 1,080,000 personal care and companionship at USD 252,000, caregiver corps training and stipends at USD 134,400 and digital and coordination infrastructure at USD 45,000 among other support mentioned above. In-kind support in the form of food supplies, transport vehicles, hygiene materials, or digital equipment would also be received with gratitude and directed to program use.

OBOMOCARE commits to full financial transparency, quarterly reporting, open audit access, beneficiary impact documentation, and regular communication with all donors and partners. The organization welcomes field visits and direct engagement with beneficiaries and staff as part of any donor relationship.

The people that OBOMOCARE serves are not invisible to us. They are our neighbors, our community members, and our fellow citizens. Each one of them has a name, a story, and a right to care. OBOMOCARE exists to make that care real. We invite you to make it possible

at scale.

OBOMOCARE Community Based Organisation

Gusii Region, Kenya | Established 2020

Contact: obomocare@gmail.com

Delivering Care.